



Pharm*Achieve*

MEDICATION RECONCILIATION

Qualifying Exam Preparatory Course

COMPETENCIES COVERED IN THIS PRESENTATION...

**Providing Care:
Clinical Care**

**Providing Care:
Distribution**

**Knowledge
& Expertise**

**Communication
& Collaboration**

**Leadership &
Stewardship**

Professionalism

LEGEND

AE	Adverse Effect
BPMH	Best Possible Medication History
EHR	Electronic Health Record
EMR	Electronic Medical Record
MAR	Medication Administration Record
MedRec	Medication Reconciliation
SSTI	Skin and Soft Tissue Infection
ISMP	Institute for Safe Medication Practices
LTC	Long-Term Care
NHP	Natural Health Product
NP	Nurse Practitioner
OTC	Over-The-Counter

CONTINUITY OF CARE

Most often refers to the transition that occurs when a patient is discharged from an acute care setting to an outpatient community environment

The concept of continuity of care in medication management refers to a pharmaceutical care plan that serves the patients' needs through all stages of the care continuum from institutional care to home care to self-care. It is an essential component of quality of care. The success of continuity of care is highly dependent on the effective cooperation and communication between all components of the healthcare system



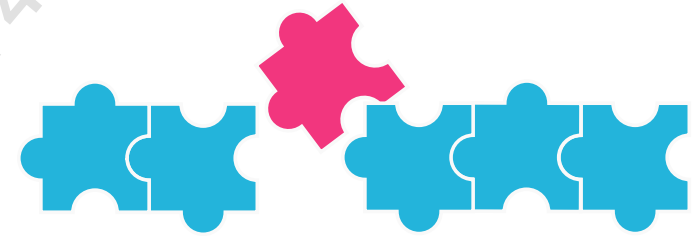
CONTINUITY OF CARE

When moving between sites of care, **patients' drug-related information is not always transferred to all care providers in a timely fashion**, resulting in fragmented patient profiles

Care is often provided without access to current and complete information

The patient may not receive the most appropriate **therapeutic regimen** for their condition and circumstances

May result in sub-par response, adverse drug events, increased use of healthcare resources and increased costs

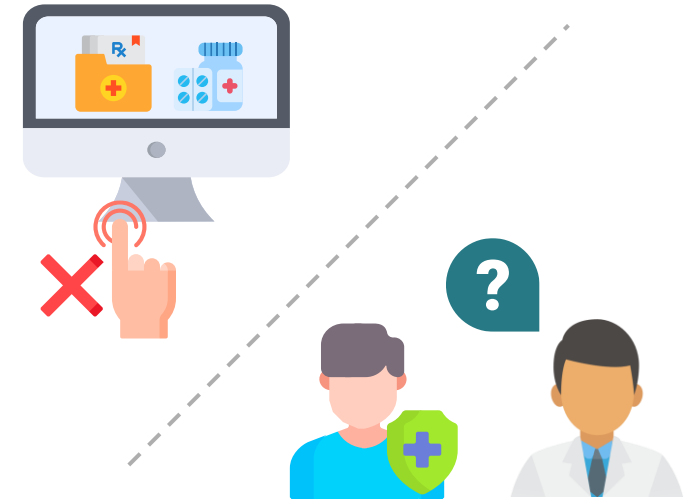


Preventable through
Medication Reconciliation

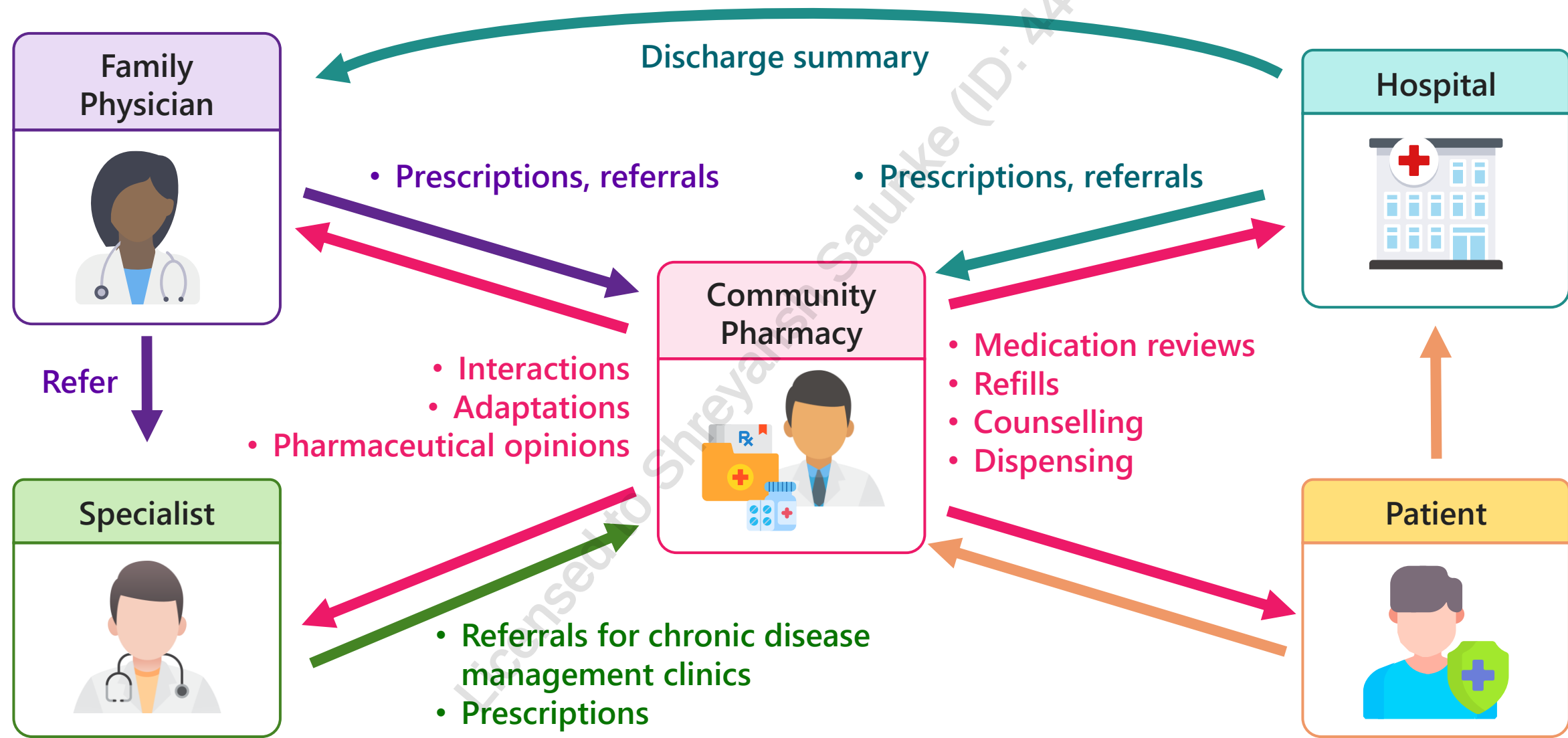
CONTINUITY OF CARE

- Patients' drug therapy is often modified during hospitalization
- On discharge, a **medical discharge summary** is usually sent to the community primary care physician who will assure continuity of care for the patient
 - **Changes in drug therapy during hospitalization are not always explained in the discharge summary**

The community pharmacist does **not** have access to this information even though they are usually the first health professional the patient sees after leaving the hospital. Due to a lack of information, the **community pharmacist is unaware** of the final diagnosis of the patient, of the therapeutic rationale for new medications and of the reason for changes made in the previous drug regimen



CURRENT MODEL



WHAT IS A MEDICATION RECONCILIATION (MEDREC)?

Good afternoon Mrs. Doe, are you ready for your medication reconciliation?

What is a medication reconciliation?



WHAT IS A MEDICATION RECONCILIATION (MED REC)?

- 
- Process of creating the most accurate list of all possible medications a patient is currently taking
 - ✓ Drug name
 - ✓ Indication
 - ✓ Dose
 - ✓ Frequency
 - ✓ Route
 - Completed ideally within 24 hours from admission to hospital & transfer
 - Ensure medication history is consistent across all transitions of care

MEDICATION RECONCILIATION

It is a formal process comparing:

An accurate and comprehensive medication history from the patient and other sources (called the **Best Possible Medication History**)



VERSUS



Medications prescribed at Admission, Transfer and Discharge

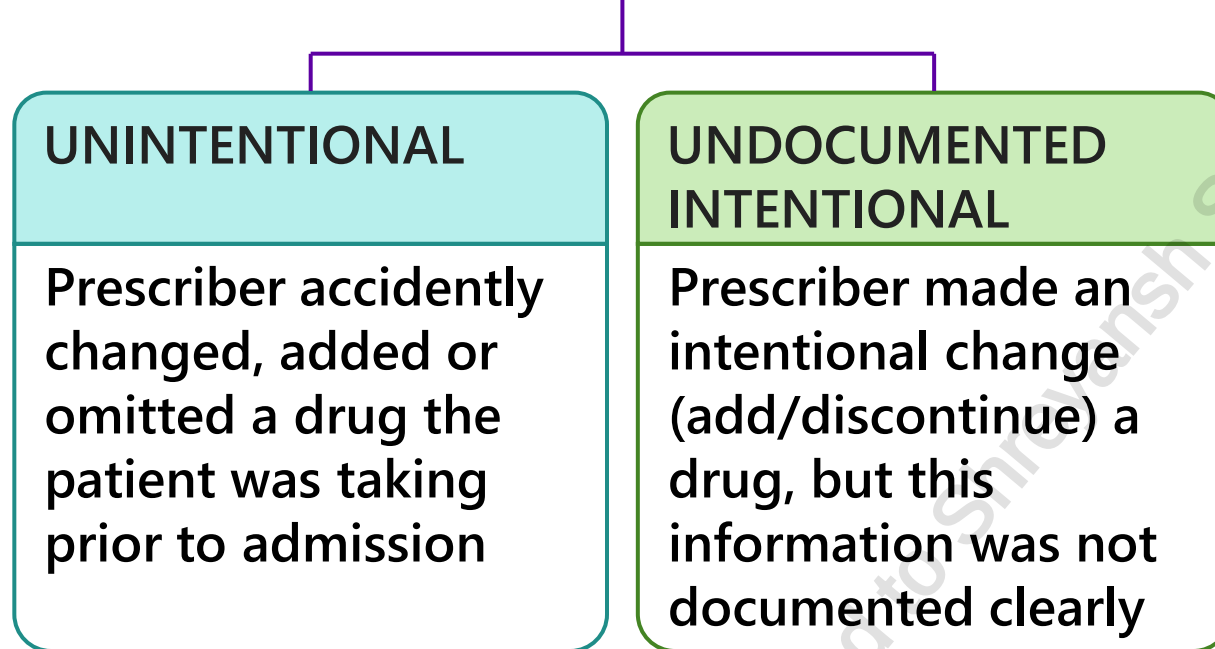
Discrepancies are identified and brought to the attention of the healthcare team



Med Rec is a system-level approach to reducing harm. The goals of Med Rec are to **reduce adverse drug events, improve patient safety and quality of service**

WHY ARE MEDRECS IMPORTANT

- ✓ Reduces *preventable* drug-related adverse events
- ✓ Reduces risk of **DISCREPANCIES**



WHAT TYPE OF DISCREPANCY IS THIS SCENARIO?

A patient was admitted to the hospital for total hip replacement surgery. A few days after the surgery, the patient started experiencing chills, sleep disturbances, and shock-like sensations. The patient's wife told the nurse before admission that the patient was taking venlafaxine, and it had not been ordered for the patient while in the hospital. The medication was then ordered and the patient started to feel better.

UNINTENTIONAL
DISCREPANCY



UNDOCUMENTED
INTENTIONAL

WHAT IS THE MEDREC PROCESS?

- 1 Best Possible Medication History (BPMH)
- 2 Reconcile the BPMH with the current admission, transfer or discharge medication orders

MEDREC PROCESS

WHAT IS THE MEDREC PROCESS?

- 1 Best Possible Medication History (BPMH) 
 - 2 Reconcile the BPMH with the current admission, transfer or discharge medication orders
- Gathering a complete list of drugs a patient is currently taking
 - Reliable sources of information
 - Community pharmacy
 - Drug vials*
 - Family physician
 - Patient and family interview
 - Insurance claims data
 - Hospital data (from prior visit)
 - BPMH be completed by a pharmacy technician, nurse, pharmacist

*Patients should be advised to bring in all drug vials from home

BEST POSSIBLE MEDICATION HISTORY

A **Best Possible Medication History (BPMH)** is a history created using:

- 1) A systemic process of interviewing the patient/family
- 2) A review of at least one other reliable source of information to obtain and verify all a patient's medication use
- 3) Complete documentation including drug name, dosage, route and frequency



The BPMH is a “**snapshot**” of the patient's actual medication use, which may be different from what was contained in their records → Patient involvement is vital

BPMH

To summarize, you are currently taking potassium supplementation and no prescription medications. Also, you developed hives from amoxicillin therapy about 5 years ago, correct?



BPMH



PATIENT HISTORY:

BPMH

- Penicillin allergy (hives, *5 years ago*)
- Home medications include potassium chloride 600 mg daily (*initiated 2 years ago*)

LAB RESULTS (*today*):

Na	NORMAL
K	HIGH (5.7)
Ca	NORMAL
Mg	NORMAL

ADMISSION ORDERS:

Piperacillin tazobactam for severe SSTI

ISMP TOP 10 PRACTICAL TIPS

1. **Be proactive**
 - Gather as much information as possible prior to seeing the patient
2. **Prompt questions about non-prescription categories**
 - Over-the-counter drugs, vitamins, supplements, and herbals
3. **Prompt questions about unique dosage forms**
 - Eye drops, inhalers, patches, sprays and injections
4. **Do not assume patients are taking medications according to prescription vials**
5. **Use open-ended questions**
6. **Use medical conditions as a trigger to prompt consideration of appropriate common medications**
7. **Consider patient adherence with prescribed regimens**
 - Verify with dispensing records
8. **Validate with at least 2 sources of information**
9. **Obtain community pharmacy contact information**
 - Inquire about use of multiple pharmacies
10. **Use a systematic BPMH trigger sheet**

MEDREC PROCESS

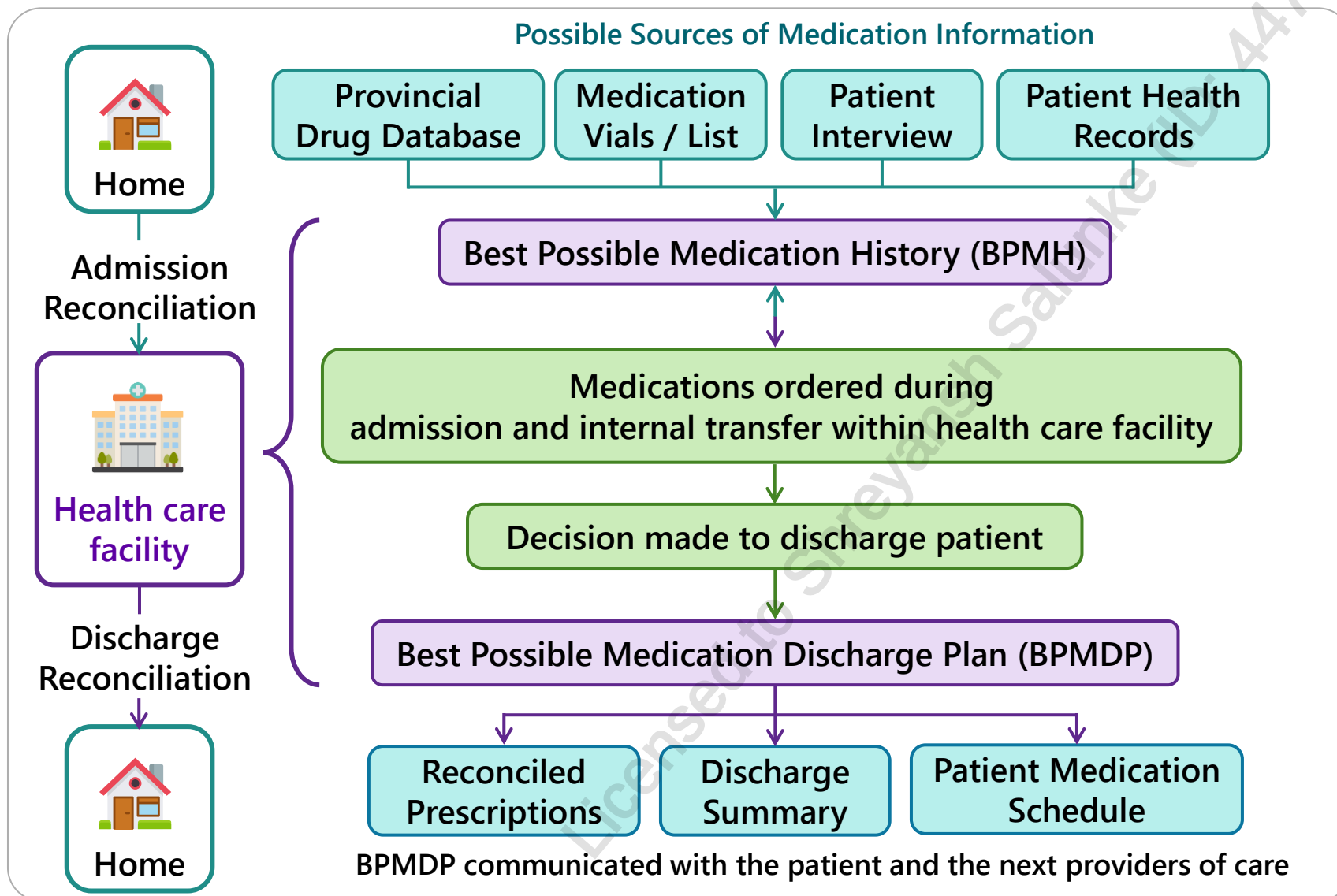
WHAT IS THE MEDREC PROCESS?

1 Best Possible Medication History (BPMH)

2 Reconcile the BPMH with the current admission, transfer or discharge medication orders 

- Identifies/resolves discrepancies
- Minimizes risk of preventable medication errors
- Opportunity to communicate changes to healthcare team
- Completed by pharmacist or physician to evaluate current therapy

MEDICATION RECONCILIATION



Medication reconciliations should be performed whenever a patient moves from one setting to another

Within the hospital setting this includes at:

- Admission
- Internal transfer
- Discharge

OPTIMIZING MEDREC USE TO REDUCE PREVENTABLE DRUG ERRORS

- Educate patients and caregivers to keep up-to-date records of medical conditions

NOT ONLY MEDICATION RECORDS

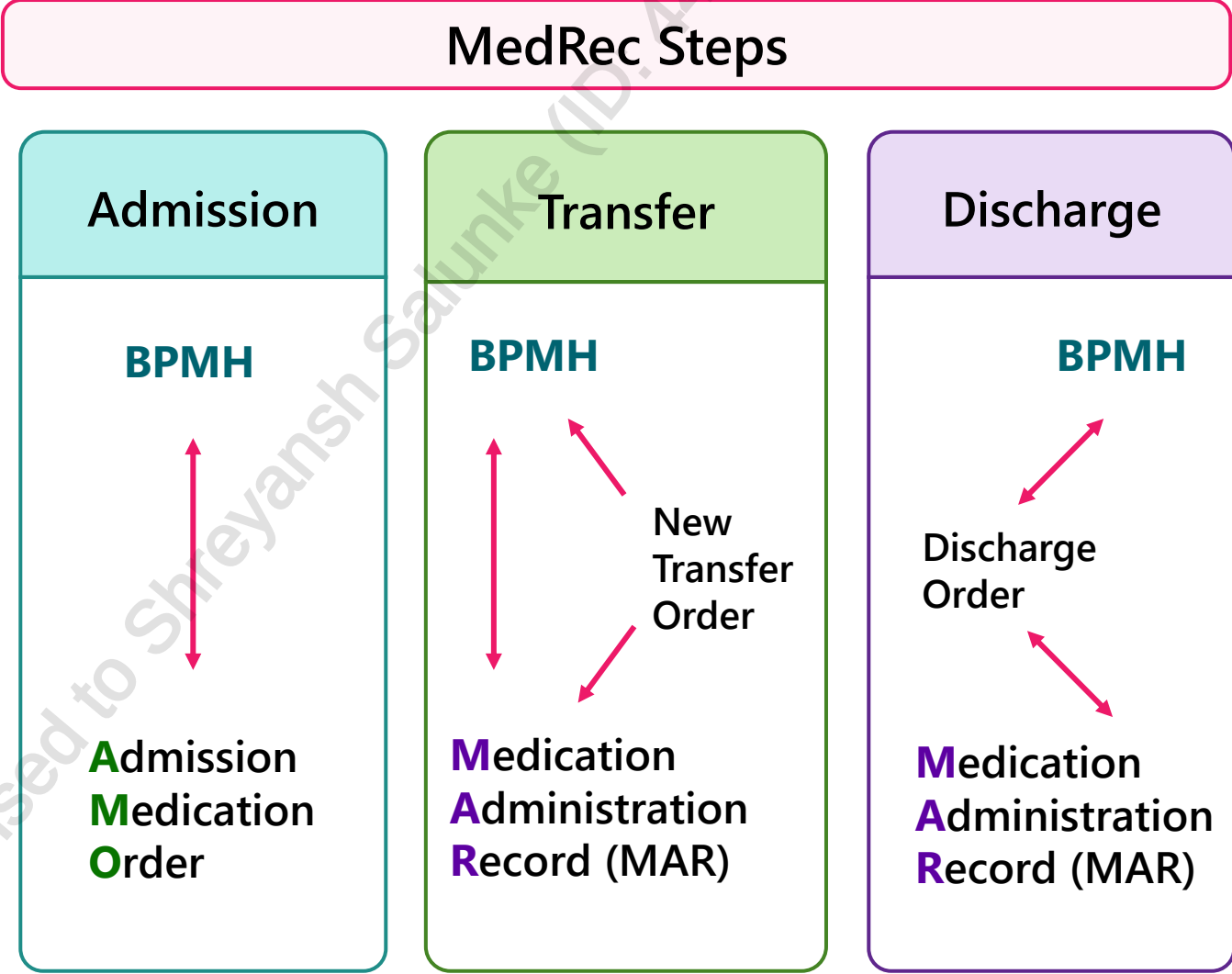
- Prescription drugs
- Over-The-Counter (OTC) products
- Natural Health Products (NHPs)
- Medical Conditions
- Allergies
- Social history

*What if a smoker is
admitted into hospital?*

- Encourage community pharmacy medication reviews prior to planned hospital admission and after discharge (*all transitions of care*)
- Encourage patients to fill all medications at one community pharmacy

WHEN SHOULD MEDRECS BE PERFORMED IN CLINICAL PRACTICE?

- At ALL transitions of care
- When changes are made to a patient's medical history (*lab values, allergies, diagnosis*)
- When changes are made to a patient's medications



WHERE SHOULD MEDRECS BE IMPLEMENTED

Hospitals

LTC homes

Ambulatory clinics

Family physician / NP offices

Community pharmacy

SAMPLE MEDREC FORM



PRACTICE CASE

John enters the pharmacy to pick up a refill on his metformin.

While chatting with him you find out that he has an upcoming knee replacement surgery in a couple of weeks.

What would you like to do next?

- ☐ Say goodbye
- ☐ Offer to do a BPMH
- ☐ Wish John good luck with his surgery

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What sources can you use for the BPMH?

- ☐ Pharmacy List
- ☐ Talk to John
- ☐ Med Vials

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- ☒ Talk to John
- ☒ Med Vials

PRACTICE CASE

Which medications would you transfer over to the BPMH?

Medication	Instructions	Dose	Quantity	Days Supply	Refills
Metformin	Take 2 tablets BID	500 mg	400	100	2
Methylprednisolone acetate	Take to your physician for injection	80 mg	2	90	0
Sitagliptin	Take 1 tablet daily	100 mg	100	100	2
Amoxicillin-clavulanic acid	Take 1 tablet BID	875 mg/ 125 mg	14	7	0
Salbutamol	Take 1-2 puffs QID PRN	100 mcg	1	20 (filled 6 months ago)	0
Celecoxib	Take 1 capsule BID	100 mg	200	100	1
Rosuvastatin	Take 1 tablet daily	10 mg	100	100	3
Perindopril	Take 1 tablet daily	4 mg	100	100	2

PRACTICE CASE

Which medications would you transfer over to the BPMH?

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PRACTICE CASE

What medication would you like to ask John about?

Metformin

Methylprednisolone acetate

Sitagliptin

Celecoxib

Rosuvastatin

Perindopril

Antibiotic + inhaler

PRACTICE CASE

In the EHR, you record the information of metformin as:

- ☐ 1000 mg BID
- ☐ 1000 mg BID with meals

John says the following about his metformin:

I take 2 pills with breakfast and 2 pills with dinner.

PRACTICE CASE

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John says the following about his metformin:

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PRACTICE CASE

In the EHR, you record the following information of methylprednisolone acetate:

- ☐ 80 mg q3months
- ☐ Left knee
- ☐ Last injection 3 months ago

John says the following about his methylprednisolone acetate:

I go to the ortho clinic every 3 months to get my left knee injected.

You then ask about his last injection:

My last injection was 3 months ago.

PRACTICE CASE

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You then ask about his last injection:

My last injection was 3 months ago.

PRACTICE CASE

In the EHR, you record the following information for celecoxib:

- ☐ Prescribed as 100 mg BID
- ☐ Actual dosing is 100 mg QAM PRN
- ☐ Takes approximately 3 times a week

John says the following about his celecoxib:

I only take this one whenever I feel like I need it.

When you ask about how often that is, he says:

Maybe around 3 times a week. I'll take it in the morning when I feel like it.

PRACTICE CASE

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PRACTICE CASE

Do you record these medications on the BPMH?

- ☐ Yes
- ☐ No

John says the following about the antibiotic and inhaler use:

I had pneumonia 6 months ago, but I don't use the inhaler anymore

PRACTICE CASE

Do you record these medications on the BPMH:

- ☐ Yes
- ☒ No

You can add these meds as a footnote to the BPMH.

It's useful to know which antibiotics a patient has used in the last 6 months.

PRACTICE CASE

What else would you like to ask John about?

OTC products

NHP

Meds from other pharmacies

Injections, eye drops, etc.?

Other possible missed meds?

Allergies

PRACTICE CASE

In the EHR, you record the acetaminophen information as:

- ☐ 325 mg Qsupper
- ☐ 500 mg Qsupper
- ☐ 650 mg Qsupper

John says the following about OTC products:

I take a Tylenol® Arthritis every day at dinner.

PRACTICE CASE

You record the acetaminophen information as:

- ☐ 325 mg Q supper
- ☐ 500 mg Q supper
- ☒ 650 mg Q supper

John says the following about OTC products:

I take a Tylenol® Arthritis every day at dinner.

PRACTICE CASE

You record the following information in the EHR:

- ☐ 3.32% diclofenac
- ☐ 2.32% topical diclofenac emulgel
- ☐ QHS
- ☐ Left knee
- ☐ Both knees

As you list out different dosage forms (injections, eye drops, inhalers, ointments), John replies:

Oh, I almost forgot about my Voltaren®! I use extra strength Voltaren® every night before I go to bed.

You clarify if it's for the left knee, John replies:

I use it on both knees.

PRACTICE CASE

You record the following information in the EHR :

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You clarify if it's for the left knee, John replies:

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PRACTICE CASE

You record the following information for trazodone in the EHR:

- ☐ Trazodone 25 mg PRN
- ☐ Trazodone 50 mg
- ☐ Not prescribed for patient
- ☐ Shares with wife
- ☐ Approximately 1 night per week

You ask John if there are any other medications that you didn't discuss, he replies:

Occasionally, I use my wife's trazodone if I'm having difficulty sleeping. I probably use it about once a week.

Upon review of the wife's profile, you see that she takes trazodone 25 mg PRN.

PRACTICE CASE

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PRACTICE CASE

Congratulations!

You have completed John's BPMH.

You make a copy of the BPMH for John so that he can take it to the hospital for his pre-admission or pre-op appointment.

RESOURCES TO USE IN PRACTICE



ISMP Canada



University of Alberta Patient Care Process



Government of Ontario | MedsCheck



Professional Pharmacy Services Guidebook 3.0

CHALLENGES TO COMPLETING MEDREC

Patient or provider level

- Responsibility for medication management (rests largely with the patient)
- Limited health literacy on part of patient
- Language and cultural barriers
- Patient's lack of awareness of importance of bringing medications and up-to-date medication lists to primary care visits
- Patient's use of multiple pharmacies
- Infrequent or periodic contact with primary care provider
- Lack of time for provider to complete MedRec
- Lack of training about conduct – how to conduct systematic, comprehensive medication histories and reconciliation processes
- Effort and dedication required for frequent modification of list

System level

- Limited access to information sources
- Unreliability or incompleteness of information sources or existence of multiple conflicting sources
- Difficulty of sharing information among providers
- Poor design of EMR/EHR systems used in primary care offices (insufficient to fully support MedRec requirements)
- Lack of availability of fully integrated provincial health record

REFERENCES

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COMMUNICATION

Qualifying Exam Preparatory Course

OBJECTIVES

- 1 Become familiar with the skills required for proper interpersonal communication
- 2 Understand how to build rapport
- 3 Provide examples of non-verbal communication
- 4 Differentiate between open and closed questions
- 5 Define audience-sensitive language
- 6 Be familiar with methods to organize the information you communicate
- 7 Avoid common communication pitfalls

FOUNDATIONS OF INTERPERSONAL COMMUNICATION

Effective dialogue as a health care professional requires the following skills



Rapport



Appropriate Language



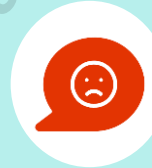
Non-Verbal Communication



Assertive vs. Passive Aggressive



Active Listening



Communication Pitfalls



Effective Questioning

RAPPORT

Relation of trust, sympathy, and mutual understanding



- ✓ Get to know your patients
- ✓ Identify yourself and your role in the pharmacy
- ✓ Display respect and empathy
- ✓ Show an understanding of the person's perspective and opinion

NON-VERBAL COMMUNICATION

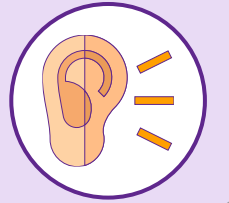
Facial expression, eye contact, gestures, posture, tone of voice



- ✓ Watch for nonverbal cues from patients e.g. expressions of pain
- ✓ Consider religious and cultural backgrounds and beliefs of the patient
- ✓ Avoid negative posture orientations (arms crossed)

ACTIVE LISTENING

More than just listening to patients!



- ✓ Verbal and non-verbal support, comfort, empathy
- ✓ Paraphrasing and summarizing what was said by the patient to ensure mutual understanding

EFFECTIVE QUESTIONING



***AVOID** leading questions*

You take ibuprofen with food, right? ❌

How do you take ibuprofen? ✅

***AVOID** compound questions*

Do you have a headache? Body ache? ❌

Are you experiencing any pain? ✅

***Ask** open-ended questions*

How do you feel today?

How is your pain today?

PRACTICE CASE

What medication would you like to ask John about?

Metformin

Methylprednisolone acetate

Sitagliptin

Celecoxib

Rosuvastatin

Perindopril

Antibiotic + inhaler

How would you ask John about his knee injections?

- **Avoid** leading questions
- **Avoid** compound questions
- **Use** open ended questions

APPROPRIATE LANGUAGE

- ✓ **AVOID** medical jargon
- ✓ Clearly explain medical concepts in lay terms



This drug will prevent gastric acid reflux from irritating your esophagus by reducing the amount of acid formed



How would you make this patient friendly?

APPROPRIATE LANGUAGE

- ✓ **AVOID** medical jargon
- ✓ Clearly explain medical concepts in lay terms



This drug will prevent gastric acid reflux from irritating your esophagus by reducing the amount of acid formed

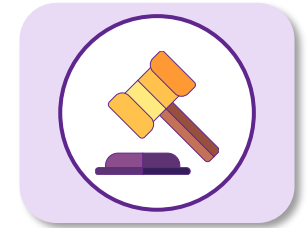


This drug will prevent the acid in your stomach from going into the tube that connects your throat to your stomach by reducing the amount of stomach acid formed



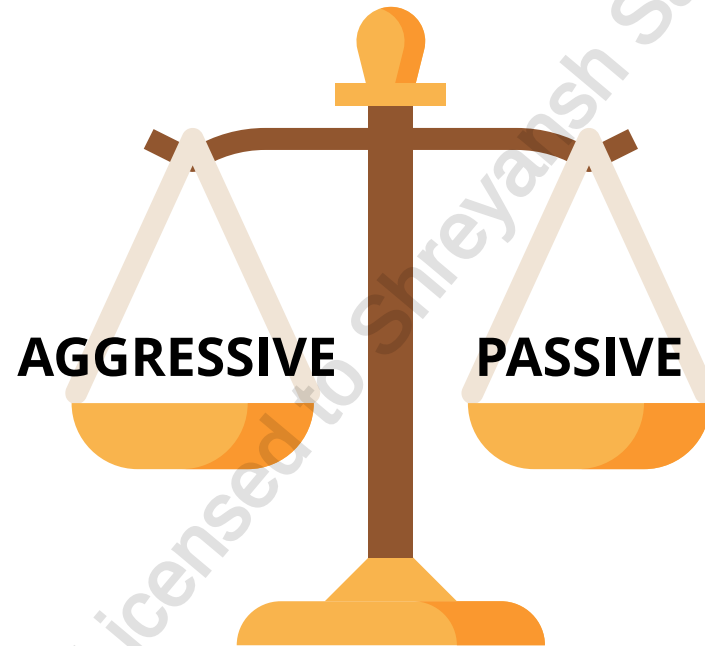
ASSERTIVE CHARACTER

- ✓ Maintains one's rights without compromising the rights of others
- ✓ Makes a recommendation that accommodates patient's beliefs



ASSERTIVE

Imposing opinions onto others, resulting in the patient pretending to agree despite being resistant to change



To give up one's rights in respect of others, giving the impression that the person lacks confidence

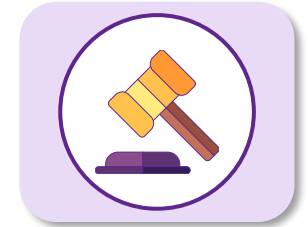
ASSERTIVE CHARACTER

Verbal

- Express needs in honest, tactful and direct ways
- "I" statements

Non-Verbal

- Direct eye contact
- Confident tone of voice
- Empathy

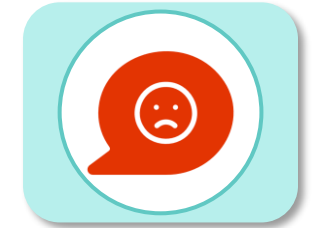


More satisfying and honest relationships

- *"What can we both agree on?"*
- *"What are your thoughts on this"*

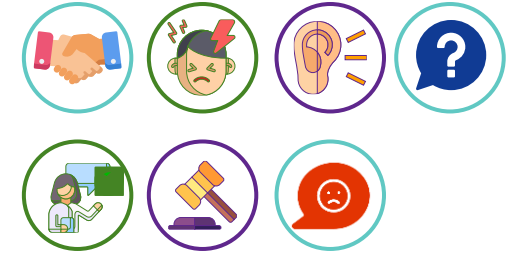
COMMUNICATION PITFALLS

- ✓ **AVOID** providing personal opinions
- ✓ **AVOID** interrupting, overpowering, lecturing patients



STRUCTURED APPROACH

Ensure that there is a clear order to the conversation
(e.g. SCHOLAR/HAMS in OTC counselling, SBAR during
interaction with another healthcare provider)



S Situation

B Background

A Assessment

R Recommendation

*BT has been experiencing dizziness
throughout the day for the past 3 days.*

*He is a 60-year-old male taking perindopril
8 mg PO daily for hypertension.*

*His blood pressure reading in the pharmacy
this morning was 95/65 mmHg.*

*I recommend decreasing his dose to 4 mg
PO daily with a re-evaluation in a week.*



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PATIENT CARE PROCESS

Qualifying Exam Preparatory Course

OBJECTIVES

- ☐ Know how to apply the following in clinical practice:
 - ☐ HAMS
 - ☐ SCHOLAR
 - ☐ QuEST
 - ☐ Prime questions
 - ☐ Documentation
 - ☐ Patient care process
 - ☐ Care plan
 - ☐ Follow-up

PATIENT INTERVIEW PROCESS

HAMS – Patients' basic medical background

H	History of conditions	• What medical conditions do you have?
A	Allergies	• Do you have any allergies?
M	Medications	• Do you take any prescription medications? • Do you take any over-the-counter medications? • Do you take any natural health products or herbals?
S	Social history	• Do you drink alcohol? What is your weekly alcohol intake? • What is your daily caffeine intake? • Do you smoke? • Are you pregnant/trying to become pregnant, or breastfeeding? • Do you use illicit/recreational drugs?

PATIENT INTERVIEW PROCESS

HAMS – Patients' basic medical background

H	History of conditions	• What medical conditions do you have?
A	Allergies	• Do you have any allergies?
M	Medications	• Do you take any medications? • Do you take over-the-counter medications? • Do you use natural health products or herbals?
S	Social history	• Do you drink alcohol? What is your weekly alcohol intake? • What is your daily caffeine intake? • Do you smoke? • Are you pregnant/trying to become pregnant, or breastfeeding? • Do you use illicit/recreational drugs?

Always do HAMS!

PATIENT INTERVIEW PROCESS

SCHOLAR (provides a comprehensive picture of the chief complaint)

S	Symptoms	What symptoms are you experiencing?
C	Characteristics	What are the symptoms like? Is it changing?
H	History	Has this happened before? What has been done so far?
O	Onset	When did this start?
L	Location	Where is the problem?
A	Aggravating factors	What makes your symptoms worse?
R	Remitting factors	Is there anything that improves your symptoms?

OPQRST is another term
(more commonly used
for pain assessment)

Onset

Provoking/Palliating

Quality

Region/Radiating

Severity

Time

PATIENT INTERVIEW PROCESS

QuEST – self care triage & counselling

Qu

Quickly and accurately assess the patient (*SCHOLAR/HAMS*)

E

Establish whether the patient is eligible for self-care

S

Suggest appropriate self-care measures (non-pharmacological and pharmacological)

T

Talk with the patient (counselling): *4As+W*

- *Action* of medication
- *Administration* schedule
- *Adverse effects* (2-3 common, 1 rare) and how to manage
- *What to expect* from treatment
- *Appropriate follow-up*

PATIENT INTERVIEW PROCESS

NEW PRESCRIPTION DROP-OFF

1. What were you told this medication is for?
✔ *Ensures correct indication*
2. Do you know how to take the medication?
✔ *Patient awareness of dosing regimen*
3. What were you told to expect?
✔ *Discussion of adverse effects, and monitoring parameters*

3 PRIME QUESTIONS + HAMS

PATIENT INTERVIEW PROCESS

REFILL PRESCRIPTIONS

1. What do you take this medication for?
✓ *Ensures correct indication*
2. How do you take this medication?
✓ *Addresses adherence*
3. Have you noticed any results?
4. Are you experiencing any new symptoms?
✓ *Addresses adverse effects*

4 PRIME QUESTIONS + Are there any changes in your medical history?

STRUCTURED COUNSELLING

NEW PRESCRIPTION PICK-UP | OTC COUNSELLING

1. WHAT DOES THE PATIENT ALREADY KNOW ABOUT THE MEDICATION?

- ☐ *3 prime questions + HAMS*

2. GENERAL DRUG INFO

- ☐ *Generic and brand name*
- ☐ *Indication*
- ☐ *Time to therapeutic effect*
- ☐ *Adverse effects*
- ☐ *How to manage adverse effects*
- ☐ *Food-drug interactions*
- ☐ *Drug-drug interactions*
- ☐ *Storage*

3. PATIENT-SPECIFIC DRUG INFO

- ☐ *Strength of medication*
- ☐ *Administration (dosing schedule)*
- ☐ *Maximum daily dose*
- ☐ *Missed doses*
- ☐ *Quantity filled, number of repeats remaining*

4. OTHER

- ☐ *Non-pharmacological measures to manage condition*
- ☐ *Follow-up with pharmacist*
- ☐ *Follow-up with MD*

PATIENT CARE PROCESS

Individualized, logical, and systematic patient assessments/evaluations and therapeutic recommendations

1 Patient Assessment

2 Care plan

3 Follow-up/evaluation

PATIENT ASSESSMENT

1. Establish therapeutic relationship

- Partnership between provider and patient to optimize drug therapy and overall health

2. Elicit information – HAMS/SCHOLAR/PRIME QUESTIONS

- Chief concern?
- Demographics?
- Medications?
- Medical history?

3. Pharmacotherapy workup

- Requires drug therapy? Prescription or OTC?
- Current drug therapy problem?
- Non-pharmacological recommendations?
- Is referral required?

PATIENT ASSESSMENT

3. Pharmacotherapy workup – IESA
- Is there a DTP?

I INDICATION

E EFFICACY

S SAFETY

A ADHERENCE

PATIENT ASSESSMENT - IESA

I	Indication	- Is medication indicated for each medical condition? - Is this a first-line option? - Can medication be discontinued?
E	Efficacy	- Is the medication effective? - If not, then why?
S	Safety	- Is there a risk of AEs? - Is the dose too high? [weight, age] - Drug-Drug, Drug- Food or Drug-Disease interactions?
A	Adherence	- Dosage Form/ Frequency ; Cost; Comprehending directions; Patient preferences - Is the patient able to administer this medication?

CARE PLAN

1. Establish goals of therapy

- Cure disease/condition?
- Reduce/eliminate symptoms?
- Slow progression of disease?
- Prevent a disease?
- Normalize laboratory values?

2. Select appropriate interventions + COUNSEL

- Drug therapy and/or non-pharmacological recommendations?
- Referral to another provider?

3. Determine desired or observable outcomes

- May include laboratory values

4. Schedule follow-up meeting



SHARED-DECISION MAKING:

Work with your patient to develop patient-specific goals and recommendations

FOLLOW-UP

Evaluate progress and investigate new concerns
Update care plan based on the following:

RESOLVED	• Acute condition • Goal achieved • Drug discontinuation	UNIMPROVED	• No progress • Further investigation: Switch drug • More time to see response: Continue drug
STABLE	• Chronic condition • Goal achieved • Drug continuation	WORSENERD	• Decline in health • Drug adjustment
IMPROVED	• Adequate progress • Drug continuation	FAILURE	• Goal NOT achieved despite proper dosing • New/change in drug therapy
PARTIAL IMPROVED	• Adequate progress • Drug adjustment	EXPIRED	• Patient passed away



DOCUMENT ALL PATIENT ENCOUNTERS!

PRACTICE CASE

LW, an 8-year-old male with asthma, enters the pharmacy to pick up a refill for his salbutamol inhaler. While speaking with his father, you find out that in the past 4 weeks, LW has been using his inhaler at least 5 times per week. LW has asthma symptoms approximately 3 days of the week and no nighttime awakenings.



PRACTICE CASE

Patient Assessment

I	
E	
S	
A	

PRACTICE CASE

Patient Assessment

I	Asthma
E	Salbutamol alone is not efficacious at controlling symptoms and does not protect patient from severe exacerbations.
S	Frequent use can increase risk of exacerbations and side effects
A	Overuse

PRACTICE CASE

Care Plan

Goals of Therapy	
Interventions	
Monitoring	

PRACTICE CASE

Care Plan

Goals of Therapy	Controlled asthma (e.g. daytime symptoms ≤ 2/week)
Interventions	- Recommendation was sent to MD to consider adding a low-dose inhaled corticosteroid (ICS) - Recommended patient book an appointment with MD - Patient was started on fluticasone MDI 50 mcg inhaled by mouth BID with an aerochamber - Counselled patient and father on appropriate use of ICS
Monitoring	- In 1 month, book appointment with LW and father to assess symptoms and inhaler use - Get patient to visit MD or a clinic for routine spirometry testing

PRACTICE CASE

Follow-Up

Evaluate Progress	
Any Changes Needed to Care Plan?	
Continue Monitoring	

TAKEAWAY

- MedRec: Consists of creating a BPMH and reconciling it with current admission, transfer or discharge medication orders
- MedRecs should be done at all stages of care transfer
- Practice active listening
- Avoid medical jargon and leading questions
- Use QuEST, **SCHOLAR/HAMS** and IESA to help guide therapeutic process

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CHANGE LOG

August 2025

- Minor formatting changes made throughout
- Content reviewed; no changes made

October 2025

- Added acronyms to legend

March 2026

- Slide 2: Updated NAPRA competencies
- Grammatical and formatting edits throughout
- Slide 18: Addition of supplements
- Slide 38: Updated information
- Deleted slide 64
- Slide 68: Added OPQRST
- Slide 71: Deleted (repeated information)
- Slide 83: Minor updates
- Slide 89: Updated references